

INQOVI® (DECITABINE 35 MG/CEDAZURIDINE 100 MG) FILM-COATED TABLETS

FULL PRESCRIBING INFORMATION

1 INDICATIONS AND USAGE

INQOVI is indicated for treatment of adult patients with myelodysplastic syndromes (MDS), including previously treated and untreated, de novo and secondary MDS with the following French-American-British subtypes (refractory anemia, refractory anemia with ringed sideroblasts, refractory anemia with excess blasts, and chronic myelomonocytic leukemia [CMML]) and intermediate-1, intermediate-2, and high-risk International Prognostic Scoring System groups.

INQOVI is indicated as monotherapy for the treatment of adult patients with newly diagnosed acute myeloid leukemia (AML) who are ineligible for standard induction chemotherapy.

2 DOSAGE AND ADMINISTRATION

2.1 Important Administration Information

Treatment with INQOVI should be initiated and supervised by a physician experienced in the use of anticancer therapies.

Do NOT substitute INQOVI for an intravenous decitabine product within a cycle.

Consider administering antiemetics prior to each dose to minimize nausea and vomiting [*see [Adverse Reactions \(6.1 Clinical Trials Experience\)](#)*].

Obtain liver chemistries and serum creatinine prior to initiation of treatment and repeat if liver/renal toxicities are suspected.

A delay or reduction in the dose per cycle is to be considered for patients who experience hematologic and non-hematologic toxicities [*see [Dosage and Administration \(2.3 Monitoring and Dosage Modifications for Adverse Reaction\)](#)*].

2.2 Recommended Dosage

The recommended dosage of INQOVI is 1 tablet (containing 35 mg decitabine and 100 mg cedazuridine) orally once daily on Days 1 through 5 of each 28-day cycle for a minimum of 4 cycles until disease progression or unacceptable toxicity. A complete or partial response may take longer than 4 cycles.

Instruct patients of the following:

- Take INQOVI at the same time each day.
- Swallow tablets whole. Do not cut, crush, or chew tablets.
- Do not consume food 2 hours before and 2 hours after each dose.
- Take one tablet a day for 5 days in each cycle. If the patient misses a dose within 12 hours of the time it is usually taken, instruct patients to take the missed dose as soon as possible and then to resume the normal daily dosing schedule. If the patient misses a dose by more than 12 hours, the patient should wait and take the missed dose the following day at the usual time. Extend the dosing period by one day for every missed dose to complete 5 daily doses for each cycle.
- Do not take an additional dose if vomiting occurs after INQOVI administration but continue with the next schedule dose.

INQOVI is a cytotoxic agent. Follow applicable special handling and disposal procedures.

2.3 Monitoring and Dosage Modifications for Adverse Reactions

Hematologic Adverse Reactions

Obtain complete blood cell counts prior to initiating INQOVI and before each cycle. Delay the next cycle if absolute neutrophil count (ANC) is less than 1,000/ μ L and platelets are less than 50,000/ μ L in the absence of active disease. Monitor complete blood cell counts until ANC is 1,000/ μ L or greater and platelets are 50,000/ μ L or greater [see [Warnings and Precautions \(5.1 Myelosuppression\)](#)].

In the absence of active disease:

- If hematologic recovery occurs (ANC at least 1,000/ μ L and platelets at least 50,000/ μ L) within 2 weeks of achieving remission, continue INQOVI at the same dose.
- If hematologic recovery does not occur (ANC at least 1,000/ μ L and platelets at least 50,000/ μ L) within 2 weeks of achieving remission,
 - Delay INQOVI for up to 2 additional weeks AND
 - Resume at a reduced dose by administering INQOVI on Days 1 through 4. Consider further dose reductions in the order listed in [Table 1](#) if myelosuppression persists after a dose reduction.
 - Maintain or increase dose in subsequent cycles as clinically indicated.

Table 1: Recommended INQOVI Dose Reductions for Myelosuppression

Dose Reduction	Dosage
First	1 tablet orally once daily on Days 1 through 4
Second	1 tablet orally once daily on Days 1 through 3
Third	1 tablet orally once daily on Days 1, 3 and 5

Manage persistent severe neutropenia and febrile neutropenia with supportive treatment [*see [Warnings and Precautions \(5.1 Myelosuppression\)](#)*].

Non-Hematologic Adverse Reactions

Delay the next cycle for the following non-hematologic adverse reactions and resume at the same or reduced dose upon resolution:

- Serum creatinine 2 mg/dL or greater
- Serum bilirubin 2 times upper limit of normal (ULN) or greater
- Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) 2 times ULN or greater
- Active or uncontrolled infection

3 DOSAGE FORMS AND STRENGTHS

INQOVI tablets contain 35 mg decitabine and 100 mg cedazuridine. The tablets are biconvex, oval-shaped, film-coated, red and debossed with “H35” on one side.

4 CONTRAINDICATIONS

Hypersensitivity to the active substance or to any of the excipients.

Breastfeeding [*see [Use in Specific Populations \(8.2 Lactation\)](#)*].

5 WARNINGS AND PRECAUTIONS

5.1 Myelosuppression

Fatal and serious myelosuppression can occur with INQOVI. Based on laboratory values, new or worsening thrombocytopenia occurred in 82% of patients, with Grade 3 or 4 occurring in 76%. Neutropenia occurred in 73% of patients, with Grade 3 or 4 occurring in 71%. Anemia occurred in 71% of patients, with Grade 3 or 4 occurring in 55%. Febrile neutropenia occurred in 33% of patients, with Grade 3 or 4 occurring in 32%. Myelosuppression (thrombocytopenia, neutropenia, anemia, and febrile neutropenia) is the most frequent cause of INQOVI dose reduction or interruption, occurring in 36% of patients. Permanent discontinuation due to myelosuppression (febrile neutropenia) occurred in 1% of patients. Myelosuppression and worsening neutropenia may occur more frequently in the first or second treatment cycles and may not necessarily indicate progression of underlying MDS.

Fatal and serious infectious complications can occur with INQOVI. Pneumonia occurred in 21% of patients, with Grade 3 or 4 occurring in 15%. Sepsis occurred in 14% of patients, with Grade 3 or 4 occurring in 11%. Fatal pneumonia occurred in 1% of patients, fatal sepsis in 1%, and fatal septic shock in 1% [*see [Adverse Reactions \(6.1 Clinical Trials Experience\)](#)*].

Obtain complete blood cell counts prior to initiation of INQOVI, prior to each cycle, and as clinically indicated to monitor response and toxicity. Administer growth factors and anti-infective therapies for treatment or prophylaxis as appropriate. Delay the next cycle and resume at the same or reduced dose as recommended [see [Dosage and Administration \(2.3 Monitoring and Dosage Modifications for Adverse Reactions\)](#)].

5.2 Embryo-Fetal Toxicity

Based on findings from human data, animal studies, and its mechanism of action, INQOVI can cause fetal harm when administered to a pregnant woman. In nonclinical studies with decitabine in mice and rats, decitabine was teratogenic, fetotoxic, and embryotoxic at doses less than the recommended human dose.

Advise pregnant women of the potential risk to a fetus. Advise females of reproductive potential to use effective contraception during treatment with INQOVI and for 6 months after the last dose. Advise males with female partners of reproductive potential to use effective contraception during treatment with INQOVI and for 3 months after the last dose [see [Use in Specific Populations \(8.1 Pregnancy, 8.3 Females and Males of Reproductive Potential\)](#)].

5.3 Differentiation syndrome

Cases of differentiation syndrome (also known as retinoic acid syndrome) have been reported in patients receiving intravenous (IV) decitabine. Differentiation syndrome may be fatal. Treatment with high dose IV corticosteroids and hemodynamic monitoring should be considered at first onset of symptoms or signs suggestive of differentiation syndrome. Temporary discontinuation should be considered until resolution of symptoms and if resumed, caution is advised.

5.4 Lactose intolerance

Inqovi contains 322.5 mg lactose monohydrate per tablet. Patients with rare hereditary problems of galactose intolerance, total lactase deficiency or glucose-galactose malabsorption should not take this medicine.

6 ADVERSE REACTIONS

The following clinically significant adverse reactions are described elsewhere in the labeling:

- Myelosuppression [see [Warnings and Precautions \(5.1 Myelosuppression\)](#)]

6.1 Clinical Trials Experience

Because clinical trials are conducted under widely variable conditions, adverse event rates observed in clinical trials of a drug cannot be directly compared with rates of clinical trials of another drug and may not reflect the rates observed in practice.

Myelodysplastic Syndrome and Chronic Myelomonocytic Leukemia

The safety of INQOVI was evaluated in a pooled safety population that includes patients enrolled in Study ASTX727-01-B and Study ASTX727-02 [see [Clinical Studies \(13 Clinical Studies\)](#)].

Patients were randomized to receive INQOVI (35 mg decitabine and 100 mg cedazuridine) orally once daily on Days 1 through 5 in Cycle 1 and decitabine 20 mg/m² intravenously on Days 1 through 5 in Cycle 2, or the reverse sequence, and then INQOVI (35 mg decitabine and 100 mg cedazuridine) orally once daily on Days 1 through 5 of each 28-day cycle in Cycles 3 and beyond. Patients were allowed to have one prior cycle of decitabine or azacitidine and there was no limit for body weight or surface area. Among the patients who received INQOVI, 61% of patients were exposed for 6 months or longer and 24% were exposed to INQOVI for greater than 1 year.

Deaths due to treatment-emergent adverse events occurred in 6% of patients, most often from sepsis (1%), septic shock (1%), pneumonia (1%), respiratory failure (1%), and one case each of cerebral hemorrhage and sudden death. Fatal and serious infections and associated complications can occur with INQOVI.

Permanent discontinuation due to an adverse event occurred in 5% of patients who received INQOVI. The most frequent adverse events resulting in permanent discontinuation were febrile neutropenia (1%) and pneumonia (1%). Myelosuppression and worsening neutropenia may occur more frequently in the first or second treatment cycles and may not necessarily indicate progression of underlying MDS.

Dose delays due to an adverse event occurred in 41% of patients who received INQOVI. The most common adverse events that led to dose delays were neutropenia (18%), febrile neutropenia (8%), thrombocytopenia (6%), anemia (5%), and leukopenia (5%).

Dose reductions due to an adverse event occurred in 19% of patients who received INQOVI. Adverse events requiring dosage reductions in > 2% of patients who received INQOVI included neutropenia (12%), anemia (3%), and thrombocytopenia (3%). Myelosuppression (thrombocytopenia, neutropenia, anemia, and febrile neutropenia) is the most frequent cause of INQOVI dose reduction or interruption, occurring in 36% of patients.

[Table 2](#) lists the treatment-emergent adverse events in the pooled safety population. Adverse drug reactions are listed according to system organ class (SOC) in MedDRA. Within each system organ class, the adverse drug reactions are ranked by frequency and then presented in order of decreasing seriousness. The corresponding frequency category for each adverse drug reaction is based on the CIOMS III convention and is defined as: very common ($\geq 1/10$); common ($> 1/100$ to $< 1/10$); uncommon ($\geq 1/1,000$ to $< 1/100$); rare ($\geq 1/10,000$ to $< 1/1,000$); very rare ($< 1/10,000$); not known (cannot be estimated from available data).

Table 2: Adverse Drug Reactions Observed with INQOVI Therapy in MDS and CMML Patients*

MedDRA SOC	MedDRA Term	MDS and CMML (N=208)			
		All CTCAE Grades		CTCAE Grade 3-4	
		%	Frequency	%	Frequency
Blood and lymphatic system disorder	Febrile neutropenia	33	very common	32	very common
	Neutropenia [†]	73	very common	71	very common
	Leukopenia [†]	87	very common	81	very common
	Thrombocytopenia [†]	82	very common	76	very common
	Anemia [†]	71	very common	55	very common
General disorders and administration site conditions	Fatigue	55	very common	5	common
	Hemorrhage ^{††}	43	very common	3	common
	Edema	30	very common	0.5	uncommon
	Pyrexia	19	very common	1	common
Gastrointestinal disorders	Mucositis	41	very common	4	common
	Nausea	40	very common	0.5	uncommon
	Diarrhea	37	very common	1	common
	Vomiting	15	very common	0	not applicable
Respiratory, thoracic, and mediastinal disorders	Dyspnea	38	very common	6	common
	Cough	28	very common	0	not applicable
Hepatobiliary disorders	Hepatic function abnormal	21	very common	3	common
Skin and subcutaneous tissue disorders	Rash	33	very common	0.5	uncommon
Nervous system disorders	Dizziness	33	very common	2	common
	Headache	30	very common	0	not applicable
Metabolism and nutritional disorders	Decreased appetite	24	very common	2	common
	Hyperglycemia [§]	54	very common	7	common

Infections and infestations	Infections (all viral, bacterial and fungal infections) [¶]	55	very common	15	very common
	Urinary tract infection	15	very common	2	common
	Upper respiratory tract infection [#]	23	very common	1	common
	Pneumonia	21	very common	15	very common
	Sepsis [‡]	14	very common	11	very common
	Cellulitis	12	very common	5	common

* Includes treatment-emergent adverse events that occurred during all cycles, including during treatment with 1 cycle of intravenous decitabine. Graded using National Cancer Institute Common Terminology Criteria for Adverse Events NCI (CTCAE v 4.0)

† These frequencies are based on lab values. Based on the frequency distribution of neutropenia, leukopenia, anemia and thrombocytopenia, 71% of the time, all of these PTs are affected together which is referred to as pancytopenia.

†† Includes hemorrhage, epistaxis

§ These frequencies are based on lab values.

¶ Excludes febrile neutropenia, pyrexia, urinary tract infection, upper respiratory tract infection, pneumonia, sepsis and cellulitis.

Includes upper respiratory tract infection, sinusitis

‡ Includes sepsis, septic shock

Clinically relevant adverse reactions in < 10% of patients who received INQOVI included:

- Acute febrile neutrophilic dermatosis (Sweet's syndrome) (1%)
- Tumor lysis syndrome (0.5%)

Acute Myeloid Leukemia

The safety of INQOVI in adult patients was evaluated in a safety population that included AML patients from one Phase 3 study (ASTX727-02-EU, N = 80).

Among the 80 patients who received INQOVI, 38% of patients were exposed for 6 months or longer and 6% were exposed to INQOVI for greater than 1 year.

Deaths due to treatment-emergent adverse events occurred in 24% of patients who received INQOVI, most often from pneumonia (8%), multiple organ dysfunction syndrome (3%), and sepsis (3%), and one case each of cardiac failure congestive, cerebral hemorrhage, febrile neutropenia, gastrointestinal perforation, general physical health deterioration, respiratory distress, septic shock, subdural hematoma, and sudden death. Fatal and serious infections and associated complications can occur with INQOVI.

Permanent discontinuation due to an adverse event occurred in 14% of patients who received INQOVI. The most frequent adverse events resulting in permanent discontinuation were pneumonia (5%) and asthenia (3%).

Treatment interruption and dose reductions due to an adverse event occurred in 48% of patients who received INQOVI. Adverse events requiring treatment interruption or dose reduction occurring in $\geq 3\%$ of patients who received INQOVI included neutropenia (13%), hematotoxicity (8%), febrile neutropenia (5%), pneumonia (5%), pleural effusion (3%), and thrombocytopenia (3%). Myelosuppression (thrombocytopenia, neutropenia, anemia, and febrile neutropenia) is the most frequent cause of INQOVI dose reduction or interruption, occurring in 19% of patients.

Table 3 lists adverse drug reactions according to system organ class (SOC) in MedDRA. Within each system organ class, the adverse drug reactions are ranked by frequency and then presented in order of decreasing seriousness. The corresponding frequency category for each adverse drug reaction is based on the CIOMS III convention and is defined as: very common ($\geq 1/10$); common ($\geq 1/100$ to $< 1/10$); uncommon ($\geq 1/1,000$ to $< 1/100$); rare ($\geq 1/10,000$ to $< 1/1,000$); very rare ($< 1/10,000$); not known (cannot be estimated from available data).

Table 3: Adverse Drug Reactions Observed with INQOVI Therapy in AML Patients

MedDRA SOC	MedDRA Term ^a	AML (N = 80)			
		All CTCAE Grades		CTCAE Grade 3-4	
		%	Frequency	%	Frequency
Infections and infestations	Pneumonia ^b	23.8	Very common	18.8	Very common
	Sepsis ^c	10.0	Very common	6.3	Common
	Cellulitis ^d	16.3	Very common	5.0	Common
	Urinary Tract Infection ^e	17.5	Very common	2.5	Common
	Viral Infection ^f	10.0	Very common	1.3	Common
	Bacteremia ^g	8.8	Common	8.8	Common
	Polyserositis	1.3	Common	1.3	Common
	Sinusitis Fungal ^h	2.5	Common	2.5	Common
	Infection	8.8	Common	6.3	Common
	Anorectal Infection ⁱ	3.8	Common	2.5	Common
	Upper Respiratory Infection ^j	6.3	Common	1.3	Common
	Oropharyngitis Fungal ^k	3.8	Common	1.3	Common
	Periodontitis ^l	5.0	Common	0	Not applicable
	Enterocolitis Viral	1.3	Common	0	Not applicable
	Bacterial Infection ^m	1.3	Common	0	Not applicable
	Ear Infection ⁿ	1.3	Common	0	Not applicable
	Respiratory Tract Infection	1.3	Common	0	Not applicable
Blood and lymphatic system disorders	Leukopenia ^o	81.3	Very common	67.5	Very common
	Thrombocytopenia ^o	73.8	Very common	67.5	Very common
	Anemia ^o	67.5	Very common	60.0	Very common
	Neutropenia ^{o,p}	41.8	Very common	41.8	Very common
	Febrile Neutropenia	28.8	Very common	26.3	Very common
Metabolism and nutrition disorders	Hyperglycemia ^{o,q}	61.1	Very common	4.2	Common
	Cerebral Hemorrhage	2.5	Common	1.3	Common

MedDRA SOC	MedDRA Term ^a	AML (N = 80)			
		All CTCAE Grades		CTCAE Grade 3-4	
		%	Frequency	%	Frequency
Nervous system disorders	Cerebral Hematoma	1.3	Common	0	Not applicable
	Subdural Hematoma	1.3	Common	0	Not applicable
	Dizziness ^r	7.5	Common	0	Not applicable
	Headache ^s	2.5	Common	0	Not applicable
Eye disorders	Eye Hemorrhage ^t	3.8	Common	0	Not applicable
Vascular disorders	Hematoma ^u	12.5	Very common	0	Not applicable
Respiratory, thoracic and mediastinal disorders	Pleural Effusion	10.0	Very common	3.8	Common
	Hypoxia	1.3	Common	1.3	Common
	Dyspnea ^v	8.8	Common	1.3	Common
	Cough ^w	6.3	Common	0	Not applicable
	Epistaxis	6.3	Common	0	Not applicable
Gastrointestinal disorders	Stomatitis ^x	10.0	Very common	1.3	Common
	Nausea	21.3	Very common	0	Not applicable
	Diarrhea ^y	13.8	Very common	0	Not applicable
	Vomiting	12.5	Very common	0	Not applicable
	Gastrointestinal Hemorrhage ^z	8.8	Common	3.8	Common
	Neutropenic Colitis	1.3	Common	1.3	Common
	Mouth Hemorrhage ^{aa}	2.5	Common	0	Not applicable
	Oral Discomfort ^{bb}	5.0	Common	0	Not applicable
Hepatobiliary disorders	Aspartate Aminotransferase increased ^{o,cc}	30.6	Very common	2.8	Common
	Alanine Aminotransferase increased ^{o,dd}	28.8	Very common	2.7	Common
	Alkaline Phosphatase increased ^{o,ee}	43.7	Very common	0	Not applicable
	Bilirubin increased ^{o,ff}	23.3	Very common	0	Not applicable
Skin and subcutaneous tissue disorders	Rash ^{gg}	5.0	Common	0	Not applicable
	Petechiae ^{hh}	2.5	Common	0	Not applicable
Renal and urinary disorders	Hematuria	1.3	Common	1.3	Common
General disorders and administration site conditions	Pyrexia	23.8	Very Common	1.3	Common
	Fatigue ⁱⁱ	27.5	Very Common	7.5	Common

^a The corresponding frequency category for each adverse drug reaction is based on the CIOMS III convention

^b Grouped terms include pneumonia, atypical pneumonia, bronchitis, lung infection

^c Grouped terms include sepsis, septic shock, systemic candida, urosepsis

^d Grouped terms include cellulitis, catheter site cellulitis, cellulitis staphylococcal, erythema, folliculitis, fungal, skin infection, furuncle, skin candida, subcutaneous abscess, skin infection

- e Grouped terms include bacteriuria, cystitis, dysuria, Escherichia urinary tract infection, urinary tract infection, urinary tract infection bacterial, urinary tract infection enterococcal
- f Grouped terms include corona virus infection, coronavirus test positive infection, herpes virus infection, herpes simplex, herpes zoster, influenza, influenza a virus test positive, influenza like illness, oral herpes, parainfluenzae virus infection, respiratory syncytial virus infection, rhinovirus infection, varicella, varicella zoster virus infection, viral infection
- g Grouped terms include bacteremia, enterococcal bacteremia, Escherichia bacteremia, klebsiella bacteremia, pseudomonal bacteremia, staphylococcal bacteremia, streptococcal bacteremia
- h Grouped terms include sinusitis aspergillus, sinusitis fungal
- i Grouped terms include anorectal infection, anal abscess, perirectal abscess
- j Grouped terms include laryngitis, nasal congestion, nasopharyngitis, pharyngeal erythema, pharyngitis, sinusitis, upper respiratory tract infection, viral upper respiratory tract infection
- k Grouped terms include oral candidiasis, oropharyngeal candidiasis
- l Grouped terms include gingival discomfort, gingival pain, gingival swelling, noninfective gingivitis, periodontal disease, periodontitis, tooth abscess, tooth infection
- m Grouped terms include bacterial infection, bacterial test positive, Enterobacter infection, enterococcal infection, Escherichia infection, staphylococcal infection ear infection,
- n Grouped terms include otitis externa, otitis media
- o Based on laboratory values
- P Neutrophils decreased (n = 79)
- q Hyperglycemia (n = 72)
- r Grouped terms include dizziness and postural dizziness
- s Grouped terms include headache, migraine and sinus headache
- t Grouped terms include eye hemorrhage, conjunctival hemorrhage, retinal hemorrhage, scleral hemorrhage, periorbital hemorrhage
- u Grouped terms include hematoma, extravasation of blood
- v Grouped terms include dyspnea, dyspnea exertional, tachypnoea, wheezing
- w Grouped terms include cough, productive cough, upper-airway cough syndrome
- x Grouped terms include stomatitis, anginal bullosa hemorrhagica, aphthous ulcer, tongue ulceration, lip ulceration, mouth swelling, mouth ulceration, oral mucosal blistering, oral mucosal erythema, mucosal ulceration, abscess oral, mucosal infection, pharyngeal ulceration
- y Grouped terms include diarrhea, faces soft
- z Grouped terms include anal hemorrhage, hemorrhoidal hemorrhage, gastrointestinal hemorrhage, melaena, rectal hemorrhage, gastric hemorrhage, hematochezia, intra-abdominal hematoma, upper gastrointestinal hemorrhage
- aa Grouped terms include mouth hemorrhage, gingival bleeding
- bb Grouped terms include gingival swelling, glossitis, oral discomfort, toothache, oropharyngeal discomfort, oropharyngeal pain, gingival discomfort, gingival pain, oral pain, oral disorder
- cc Aspartate Aminotransferase increased (n = 72)
- dd Alanine Aminotransferase increased (n = 73)
- ee Alkaline Phosphatase increased (n = 71)
- ff Bilirubin increased (n = 73)
- gg Grouped terms include rash, ear pruritus, pruritus, pruritus generalized, rash erythematous, rash maculo-papular, rash popular, rash pruritic, urticaria
- hh Grouped terms include petechiae, ecchymosis, increased tendency to bruise, purpura
- ii Grouped terms include fatigue, asthenia, malaise, lethargy CTCAE = Common Terminology Criteria for Adverse Events

6.2 Post marketing Experience

The following adverse reactions have been identified during post approval use of intravenous decitabine. Because these reactions are reported voluntarily from a population of uncertain size, it is not always possible to reliably estimate their frequency or establish a causal relationship to drug exposure.

Blood and Lymphatic System Disorders: Differentiation syndrome (very rare)

Respiratory, Thoracic and Mediastinal Disorders: Interstitial lung disease

Cardiac Disorders: Cardiomyopathy

7 DRUG INTERACTIONS

7.1 Effects of Other Drugs on INQOVI

Drugs Inhibited by Cytidine Deaminase

Because decitabine is a substrate for the cytidine deaminase (CDA) enzyme, which metabolizes decitabine resulting in inactive deaminated form, medicinal products inhibiting cytidine deaminase should be avoided with INQOVI as coadministration may result in increased decitabine exposures.

7.2 Effects of INQOVI on Other Drugs

Drugs Metabolized by Cytidine Deaminase

Cedazuridine is an inhibitor of the cytidine deaminase (CDA) enzyme. Coadministration of INQOVI with drugs that are metabolized by CDA (i.e. cytarabine, gemcitabine, azacitidine, zalcitabine, zidovudine, telbivudine, didanosine, stavudine, lamivudine, abacavir, emtricitabine, apricitabine, tenofovir, adefovir, idoxuridine, entecavir, trifluridine, vidarabine) may result in increased systemic exposure with potential for increased toxicity of these drugs [see [Clinical Pharmacology \(11.3 Pharmacokinetics\)](#)]. Avoid coadministration of INQOVI with drugs that are metabolized by CDA.

In vitro studies

Based on in vitro studies, CYP450-mediated drug-drug interactions are unlikely with the active ingredients in INQOVI (decitabine or cedazuridine) and INQOVI is not expected to affect P-gp mediated transport of co-administered medicinal products [see [Clinical Pharmacology \(11.3 Pharmacokinetics\)](#)].

Food

In a crossover food effect study in 16 patients, administration of INQOVI with a high-fat, high-calorie meal reduced the overall decitabine exposure (AUC) by approximately 40% and $C_{\max}$ by 54%. Cedazuridine time to maximum concentration ($T_{\max}$) was slightly delayed but its systemic exposure was not affected by the meal.

8 USE IN SPECIFIC POPULATIONS

8.1 Pregnancy

Risk Summary

Based on findings from human data, animal studies, and its mechanism of action [see [Clinical Pharmacology \(11.1 Mechanism of Action\)](#)], INQOVI can cause fetal harm when administered to a pregnant woman. A single published case report of intravenous decitabine use throughout the first trimester during pregnancy describes adverse developmental outcomes, including major birth defects (structural abnormalities). In animal reproduction studies, intravenous administration of decitabine to pregnant mice and rats during organogenesis at doses approximately 7% of the recommended human dose on a body surface area (mg/m^2) basis caused adverse developmental outcomes, including increased embryo-fetal mortality, alterations to growth, and structural abnormalities (see [Data](#)). Advise pregnant women of the potential risk to a fetus.

The estimated background risk of major birth defects and miscarriage for the indicated population is unknown. All pregnancies have a background risk of birth defect, loss, or other adverse outcomes. In the U.S. general population, the estimated background risk of major birth defects and miscarriage in clinically recognized pregnancies is 2% to 4% and 15% to 20%, respectively.

Data

Human Data

There are no available data on INQOVI use in pregnant women.

A single published case report of intravenous decitabine pregnancy exposure in a 39-year-old woman with a hematologic malignancy described multiple structural abnormalities after 6 cycles of therapy in the 18th week of gestation. These abnormalities included holoprosencephaly, absence of nasal bone, mid-facial deformity, cleft lip and palate, polydactyly, and rocker-bottom feet. The pregnancy was terminated.

Animal Data

No reproductive or developmental toxicity studies have been conducted with INQOVI or cedazuridine.

In utero exposure to decitabine causes temporal-related defects in the rat and/or mouse, which include growth suppression, exencephaly, defective skull bones, rib/sternabrae defects, phocomelia, digit defects, micrognathia, gastroschisis, and micromelia. Decitabine inhibits proliferation and increases apoptosis of neural progenitor cells of the fetal central nervous system (CNS) and induces palatal clefting in the developing murine fetus. Studies in mice have also shown that decitabine administration during osteoblastogenesis (Day 10 of gestation) induces bone loss in offspring.

In mice exposed to single intraperitoneal decitabine injections (0, 0.9 and 3.0 mg/m^2 , approximately 2% and 7% of the recommended daily clinical dose, respectively) over gestation Days 8, 9, 10 or 11, no maternal toxicity was observed, but reduced fetal survival was observed after treatment at 3 mg/m^2 and decreased fetal weight was observed at both dose levels. The 3 mg/m^2 dose elicited characteristic fetal

defects for each treatment day, including supernumerary ribs (both dose levels), fused vertebrae and ribs, cleft palate, vertebral defects, hind-limb defects, and digital defects of fore- and hind-limbs.

In rats given a single intraperitoneal injection of 2.4, 3.6 or 6 mg/m² decitabine (approximately 5, 8, or 13% the daily recommended clinical dose, respectively) on gestation Days 9 to 12, no maternal toxicity was observed. No live fetuses were seen at any dose when decitabine was injected on gestation Day 9. A significant decrease in fetal survival and reduced fetal weight at doses greater than 3.6 mg/m² was seen when decitabine was given on gestation Day 10. Increased incidences of vertebral and rib anomalies were seen at all dose levels, and induction of exophthalmia, exencephaly, and cleft palate were observed at 6.0 mg/m². Increased incidence of foredigit defects was seen in fetuses at doses greater than 3.6 mg/m². Reduced size and ossification of long bones of the forelimb and hind-limb were noted at 6 mg/m².

The effect of decitabine on postnatal development and reproductive capacity was evaluated in mice administered a single 3 mg/m² intraperitoneal injection (approximately 7% the recommended daily clinical dose) on Day 10 of gestation. Body weights of males and females exposed in utero to decitabine were significantly reduced relative to controls at all postnatal time points. No consistent effect on fertility was seen when female mice exposed in utero were mated to untreated males. Untreated females mated to males exposed in utero showed decreased fertility at 3 and 5 months of age (36% and 0% pregnancy rate, respectively). Follow up studies indicated that treatment of pregnant mice with decitabine on gestation Day 10 was associated with a reduced pregnancy rate resulting from effects on sperm production in the F1-generation.

8.2 Lactation

Risk Summary

There are no data on the presence of cedazuridine, decitabine, or their metabolites in human milk or on their effects on the breastfed child or milk production. Because of the potential for serious adverse reactions in the breastfed child, breastfeeding is contraindicated during treatment with INQOVI and for 2 weeks after the last dose.

8.3 Females and Males of Reproductive Potential

INQOVI can cause fetal harm when administered to a pregnant woman [see [Use in Specific Populations \(8.1 Pregnancy\)](#)].

Pregnancy Testing

Verify the pregnancy status in females of reproductive potential prior to initiating INQOVI.

Contraception

Females

Advise females of reproductive potential to use effective contraception during treatment with INQOVI and for 6 months after the last dose.

Males

Based on genotoxicity findings, advise males with female partners of reproductive potential to use effective contraception during treatment with INQOVI and for 3 months after the last dose [see [Nonclinical Toxicology \(12.1 Carcinogenesis, Mutagenesis, Impairment of Fertility\)](#)].

Infertility

Based on findings of decitabine and cedazuridine in animals, INQOVI may impair male fertility [see [Nonclinical Toxicology \(12.1 Carcinogenesis, Mutagenesis, Impairment of Fertility\)](#)]. The reversibility of the effect on fertility is unknown.

8.4 Pediatric Use

The safety and effectiveness of INQOVI have not been established in pediatric patients (aged less than 18 years).

8.5 Geriatric Use

Myelodysplastic Syndrome and Chronic Myelomonocytic Leukemia

Of the 208 patients in clinical studies who received INQOVI, 75% were age 65 years and older, while 36% were age 75 years and older. No overall differences in safety or effectiveness were observed between patients age 65 years and older, 75 years and older, and younger patients.

Acute Myeloid Leukemia

Of the 80 patients in clinical studies who received INQOVI, 39% were younger than 75 years, and 61% were 75 years and older. No overall differences in safety or effectiveness were observed between patients age 75 years and older and younger patients.

8.6 Renal Impairment

No dosage modification of INQOVI is recommended for patients with mild or moderate renal impairment (creatinine clearance [CLcr] of 30 to 89 mL/min based on Cockcroft-Gault). Due to the potential for increased adverse reactions, monitor patients with moderate renal impairment (CLcr 30 to 59 mL/min) frequently for adverse reactions. INQOVI has not been studied in patients with severe renal impairment (CLcr 15 to 29 mL/min) or end-stage renal disease (ESRD: CLcr <15 mL/min) [see [Clinical Pharmacology \(11.3 Pharmacokinetics\)](#)].

8.7 Hepatic Impairment

No adjustment of starting dosage is recommended when administering INQOVI in patients with mild hepatic impairment (total bilirubin > 1 to $\leq 1.5 \times$ ULN). INQOVI has not been studied in patients with moderate (total bilirubin > 1.5 to $3 \times$ ULN) or severe hepatic impairment (total bilirubin > $3 \times$ ULN) [see [Clinical Pharmacology \(11.3 Pharmacokinetics\)](#)].

8.8 Effects on Ability to Drive and Use Machines

Patients should be advised that they may experience undesirable effects such as fatigue and dizziness during treatment with INQOVI. Therefore, caution should be observed when driving a car or operating machinery.

9 OVERDOSE

9.1 Signs and Symptoms

Overdosage could cause increased myelosuppression, and neutropenia-related infections such as pneumonia and sepsis.

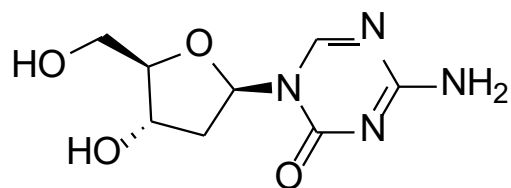
9.2 Management

There is no known antidote for overdosage with INQOVI. In the event of an overdose, patients must be closely monitored for signs or symptoms of adverse reactions and appropriate symptomatic treatment instituted.

10 DESCRIPTION

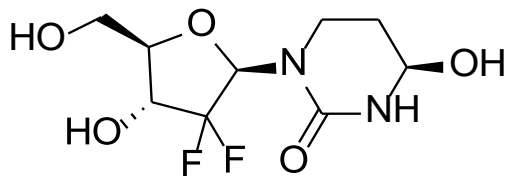
Decitabine

Decitabine is a nucleoside metabolic inhibitor. Decitabine is a white to off-white solid with the molecular formula of $C_8H_{12}N_4O_4$ and a molecular weight of 228.21 daltons. Its international union of pure and applied chemistry (IUPAC) chemical name is 4-amino-1-[(2*R*,4*S*,5*R*)-4-hydroxy-5-(hydroxymethyl)oxolan-2-yl]-1,3,5-triazin-2(1*H*)-one and it has the following structural formula:



Cedazuridine

Cedazuridine is a cytidine deaminase inhibitor. Cedazuridine is a white to off-white solid with the molecular formula of $C_9H_{14}F_2N_2O_5$ and a molecular weight of 268.21 daltons. Its IUPAC chemical name is (4*R*)-1-[(2*R*,4*R*,5*R*)-3,3-difluoro-4-hydroxy-5-(hydroxymethyl)oxolan-2-yl]-4-hydroxy-1,3-diazinan-2-one and it has the following structural formula:



INQOVI

INQOVI (decitabine and cedazuridine) tablets, for oral use contain 35 mg decitabine and 100 mg cedazuridine. The tablets are biconvex, oval-shaped, film-coated, red and debossed with “H35” on one side. Each film-coated tablet contains the following inactive ingredients: lactose monohydrate, hypromellose, croscarmellose sodium, colloidal silicon dioxide, and magnesium stearate. The film coating material contains polyvinyl alcohol, titanium dioxide, polyethylene glycol, talc, and iron oxide red.

11 CLINICAL PHARMACOLOGY

11.1 Mechanism of Action

Decitabine is a nucleoside metabolic inhibitor that is believed to exert its effects after phosphorylation and direct incorporation into DNA and inhibition of DNA methyltransferase, causing hypomethylation of DNA and cellular differentiation and/or apoptosis. Decitabine inhibits DNA methylation in vitro, which is achieved at concentrations that do not cause major suppression of DNA synthesis. Decitabine-induced hypomethylation in cancer cells may restore normal function to genes that are critical for the control of cellular differentiation and proliferation. In rapidly dividing cells, the cytotoxicity of decitabine may also be attributed to the formation of covalent adducts between DNA methyltransferase and decitabine incorporated into DNA. Non-proliferating cells are relatively insensitive to decitabine.

Cytidine deaminase (CDA) is an enzyme that catalyzes the degradation of cytidine, including the cytidine analog decitabine. High levels of CDA in the gastrointestinal tract and liver degrade decitabine and limit its oral bioavailability. Cedazuridine is a CDA inhibitor. Administration of cedazuridine with decitabine increases systemic exposure of decitabine via inhibition of first pass metabolism of decitabine in the gut and liver by CDA.

11.2 Pharmacodynamics

Decitabine induced hypomethylation both in vitro and in vivo. In patients administered the recommended dosage of INQOVI, the maximum change from baseline in the long interspersed nucleotide elements-1 (LINE-1) demethylation was observed at Day 8, with less than complete recovery of LINE-1 methylation to baseline at the end of the treatment cycle.

Based on the exposure-response analyses, a relationship between an increase in 5-day cumulative daily decitabine exposure and a greater likelihood of some adverse reactions (e.g., any grade neutropenias, thrombocytopenia) was observed in clinical studies.

In Vitro Antitumor Effects

In vitro, decitabine produced antiproliferative and differentiating effects in a panel of leukemic cell lines from murine and human origin, with IC50 values generally below 1 µM. Similarly, decitabine decreased the proliferation of many solid tumour cell lines with IC50 values from the nM to the µM range. The differentiation effects were usually observed at concentrations that did not show clear cytotoxic effects. Also, in models of normal haematopoietic cell differentiation, decitabine reduced the growth and induced differentiation characteristic of normal haematopoietic cells without inducing cytotoxicity at concentrations of 10, 50, and 100 nM.

In Vivo Effects in Tumour Models

Decitabine demonstrated dose-dependent antitumour activity in several mouse leukaemia models as well as in a rat myeloid leukaemia model, with a clear effect on survival at well-tolerated doses.

Cardiac Electrophysiology

Limited electrocardiogram (ECG) and pharmacokinetics assessments before and after dosing at the expected time of maximum concentration in plasma (T_{max}) for both decitabine (from IV or INQOVI) and cedazuridine were conducted in the Phase 3 clinical study. The assessments found no concentration dependent QTc interval prolongation for either decitabine or cedazuridine. A dedicated study to evaluate the QT prolongation potential of cedazuridine did not show any effect on any ECG parameters including at the supratherapeutic (400 mg) dose.

11.3 Pharmacokinetics

The pharmacokinetics (PK) of decitabine and cedazuridine were studied following administration of INQOVI at the recommended dosage in patients with MDS, CMML, and AML.

INQOVI achieved decitabine area under the curve (AUC) exposures equivalent to those achieved with IV infusion of decitabine at 20 mg/m² with the recommended dosage of INQOVI for 5 consecutive days [see [Clinical Pharmacology \(11.2 Pharmacodynamic\)](#)]. The geometric mean ratio (GMR) of 5-day total decitabine AUC_{0-24hr} between INQOVI and IV decitabine was 99% for patients with MDS/CMML and 100% for patients with AML (90% confidence interval [CI] 93%, 106% and 91%, 109% for MDS/CMML and AML, respectively).

An approximately dose-proportional increase in peak concentrations (C_{max}) and AUC over the dosing interval was observed for decitabine over a dose range from 20 mg to 40 mg (0.6 to 1.1 times the recommended dose) in combination with 100 mg cedazuridine, and for cedazuridine over a dose range from 40 mg to 100 mg once daily (0.4 to 1.0 times the recommended dose) in combination with 20 mg decitabine.

At steady state (achieved with the second dose), circulating plasma concentrations were typically 1.8 times and 1.1 times the Day 1 plasma concentrations for decitabine and cedazuridine, respectively.

In MDS population (highest number of subjects available; data from AML were similar), decitabine mean (%CV) AUC_{0-24hr} exposure at steady state was 189 (55%) ng•hr/mL and C_{max} was 145 (55%) ng/mL, respectively. Cedazuridine mean AUC_{0-24hr} exposure at steady state (Day 2) was 3290 (45%) ng•hr/mL and C_{max} was 349 (49%) ng/mL.

Absorption

After oral administration of INQOVI, the median T_{max} at steady state was 3 hours (range: 0.5 to 7.9) for cedazuridine and 1 hour (range: 0.3 to 3) for decitabine. When co-administered with cedazuridine, decitabine oral relative bioavailability is enhanced to achieve systemic AUC exposures seen with IV decitabine. The bioavailability of cedazuridine was 20.7% (range: 12.7% to 25.6%).

Distribution

Decitabine

Decitabine is approximately 5% bound to human plasma proteins *in vitro*. The geometric mean (CV%) of apparent volume of distribution at steady state is 417 L (54%).

Cedazuridine

Cedazuridine is approximately 35% bound to human plasma proteins *in vitro*. The geometric mean (CV%) of apparent volume of distribution for cedazuridine is 296 L (51%).

Biotransformation

Decitabine

Decitabine is metabolised primarily via deamination by cytidine deaminases and also physiochemical degradation at physiological conditions.

Cedazuridine

The primary metabolic pathway for cedazuridine is conversion to its epimer by physiochemical degradation.

Elimination

Decitabine

Following a single oral dose of INQOVI, the mean (CV%) terminal elimination half-life (T_{1/2}) of decitabine was 1.2 (23%) hours. The apparent oral clearance (CL/F) was 197 L/hr at steady state. The

main pathway of elimination for decitabine is metabolic / degradation. Metabolites and degradation products are excreted mainly renally.

Cedazuridine

Following a single oral dose of INQOVI, the mean (CV%) $T_{1/2}$ of cedazuridine was 6.3 (18%) hours. The apparent oral clearance (CL/F) was 30.3 L/hr at steady state.

The predominant elimination pathway of cedazuridine is renal, as parent drug and its epimer. Following a single oral dose of 100 mg radiolabeled cedazuridine, 46% (17.1% unchanged) of the administered dose was recovered in urine and 51% (mostly unabsorbed drug, 27% unchanged) was recovered in the faeces.

Specific Populations

Based on population PK analysis, age (32 to 90 years), sex, body weight (41 to 158 kg), and body surface area (1.3 to 2.9 m^2) did not have a clinically meaningful effect on the pharmacokinetics of decitabine or cedazuridine after dosing with INQOVI.

Renal impairment

Based on a population PK analysis of 66 patients with mild renal impairment ($\text{CrCl } 40$ to $59 \text{ mL/min/1.73 m}^2$) and 18 patients with moderate renal impairment ($\text{CrCl } 20$ to $39 \text{ mL/min/1.73 m}^2$), cedazuridine exposure increased with decreasing CrCl as compared to 129 patients with normal renal function ($\text{CrCl} \geq 60 \text{ mL/min/1.73 m}^2$). However mild to moderate renal impairment did not have a clinically meaningful effect on the pharmacokinetics of decitabine and cedazuridine after dosing with INQOVI. The effects of severe renal impairment ($\text{CrCl} < 20 \text{ mL/min/1.73 m}^2$) or end stage renal disease on the pharmacokinetics of decitabine and cedazuridine are unknown.

Hepatic impairment

Based on a population PK analysis, no clinically meaningful effect on the pharmacokinetics of decitabine or cedazuridine were observed between subjects with mild ($n = 23$) hepatic impairment (total bilirubin > 1 to $\leq 1.5 \times \text{ULN}$) relative to subjects with normal ($n = 187$) hepatic function (total bilirubin $\leq \text{ULN}$). The effects of moderate (total bilirubin > 1.5 to $3 \times \text{ULN}$) and severe hepatic impairment (total bilirubin $> 3 \times \text{ULN}$) on the pharmacokinetics of decitabine and cedazuridine are unknown.

Drug Interaction Studies

Clinical Studies

Decitabine had no clinically meaningful effect on the pharmacokinetics of cedazuridine. Cedazuridine increased the exposure of decitabine.

The coadministration of INQOVI with proton pump inhibitors had no clinically meaningful effect on exposure to decitabine or cedazuridine.

In vitro Studies

CYP Enzymes: Cedazuridine is not a substrate of cytochrome P450 (CYP) enzymes. Cedazuridine does not induce CYP1A, CYP2B6, CYP2C9, or CYP3A or inhibit CYP1A, CYP2B6, CYP2C8, CYP2C9, CYP2C19, CYP2D6, CYP2E1, or CYP3A.

Transporter Systems: Cedazuridine is not a substrate of P-glycoprotein (P-gp), MATE1, MATE2-K, OAT1, OAT3, OATP1B1, OATP1B3, OATP2B1, OCT1, or OCT2, and does not inhibit P-gp, BCRP, MATE1, MATE2-K, OAT1, OAT3, OATP1B1, OATP1B3, or OCT2.

12 NONCLINICAL TOXICOLOGY

12.1 Carcinogenesis, Mutagenesis, Impairment of Fertility

Carcinogenicity studies with decitabine, cedazuridine, or their combination have not been conducted.

INQOVI is genotoxic. Decitabine increased mutation frequency in L5178Y mouse lymphoma cells, and mutations were produced in an *E. coli* lac-I transgene in colonic DNA of decitabine-treated mice. Decitabine also caused chromosomal rearrangements in larvae of fruit flies. Cedazuridine was genotoxic in a reverse bacterial mutation assay (Ames assay) and in an in vitro chromosomal aberration study using human lymphocytes. Cedazuridine was negative for the genotoxicity assessment in three *in vivo* studies including the mouse micronucleus, Comet assay, and the Pig-A assay.

Fertility and repeat-dose toxicity studies in animals showed adverse outcomes on reproductive function and fertility. In male mice given intraperitoneal injections of 0.15, 0.3, or 0.45 mg/m² decitabine (approximately 0.3% to 1% the recommended clinical dose) 3 times a week for 7 weeks, testes weights were reduced, abnormal histology was observed, and significant decreases in sperm number were found at doses ≥ 0.3 mg/m². In females mated to males dosed with ≥ 0.3 mg/m² decitabine, pregnancy rate was reduced, and preimplantation loss was significantly increased.

Decitabine was administered orally to rats at 0.75, 2.5, or 7.5 mg/kg/day in cycles of 5-days-on/23-days-off for a total of 90 days. Low testes and epididymis weights, abnormal histology, and reduced sperm number were observed at doses ≥ 0.75 mg/kg. The dose of 0.75 mg/kg resulted in exposures in animals that were approximately 3 times the exposure in patients at the recommended clinical dose based on AUC.

Cedazuridine was administered orally to mice at 100, 300, or 1,000 mg/kg/day in cycles of 7-days-on/21-days-off for a total of 91 days. Adverse findings in male and female reproductive organs were observed at the 1,000 mg/kg dose and included abnormal histology in the testes and epididymis, reduced sperm number, and abnormal histology in the ovary. The dose of 1,000 mg/kg/day resulted in exposures in animals that were approximately 108 times the exposure in patients at the recommended clinical dose. Adverse effects in male and female reproductive organs were reversible following a recovery period.

13 CLINICAL STUDIES

Myelodysplastic Syndrome and Chronic Myelomonocytic Leukemia

Study ASTX727-01-B

INQOVI was evaluated in Study ASTX727-01-B, an open-label, randomized, 2-cycle, 2-sequence crossover study (NCT02103478) that included 80 adult patients with MDS (International Prognostic Scoring System [IPSS] Intermediate-1, Intermediate-2, or high-risk) or CMML. Patients were randomized 1:1 to receive INQOVI (35 mg decitabine and 100 mg cedazuridine) orally in Cycle 1 and decitabine 20 mg/m² intravenously in Cycle 2 or the reverse sequence. Both INQOVI and intravenous decitabine were administered once daily on Days 1 through 5 of the 28-day cycle. Starting with Cycle 3, all patients received INQOVI orally once daily on Days 1 through 5 of each 28-day cycle until disease progression or unacceptable toxicity. Randomization was stratified by IPSS risk level. Twelve (15%) of the 80 patients went on to stem cell transplantation following INQOVI treatment.

The baseline demographic and disease characteristics are shown in [Table 4](#).

Table 4: Demographics and Baseline Disease Characteristics for Study ASTX727-01-B

Characteristic	N = 80
Age	
Median (min, max) (years)	71 (32, 90)
Sex (%)	
Male	76
Female	24
Race (%)	
White	93
Black or African American	3
Asian	1
Other or Not Reported	4
ECOG Performance Score (%)	
0	44

Characteristic	N = 80
1	48
2	9
Disease Category / IPSS (%)	
MDS INT-1	44
MDS INT-2	24
MDS High-Risk	11
CMML	21
Prior HMA Therapy* (%)	
Prior Azacitidine	4
Prior Decitabine	4
Transfusion Dependence[†] (%)	
RBC Transfusion Dependence	48
Platelet Transfusion Dependence	15

* One cycle only, per the Exclusion Criteria.

[†] Defined as documentation of ≥ 2 units of transfusion within 56 days prior to the first day of study treatment.

Efficacy was established on the basis of complete response (CR) and the rate of conversion from transfusion dependence to transfusion independence. Efficacy results are shown in [Table 5](#). The median follow-up time was 24.0 months (range: 12.0 to 28.8 months) and median treatment duration was 6.6 months (range < 0.1 to 27.9).

Table 5: Efficacy Results in Patients with MDS or CMML from Study ASTX727-01-B

Efficacy Endpoint	INQOVI N = 80
Complete Response (%) (95% CI)	18 (10, 28)
Median Duration of CR - months (range)*	8.7 (1.1, 18.2)
Median Time to CR - months (range)	4.8 (1.7, 10.0)

* From start of CR until relapse or death.

Among the 41 patients who were dependent on red blood cell (RBC) and/or platelet transfusions at baseline, 20 (49%) became independent of RBC and platelet transfusions during any consecutive 56-day post-baseline period. Of the 39 patients who were independent of both RBC and platelet transfusions at baseline, 25 (64%) remained transfusion-independent during any consecutive 56-day post-baseline period.

Study ASTX727-02

INQOVI was evaluated in ASTX727-02, an open-label, randomized, 2-cycle, 2-sequence crossover study (NCT03306264) that included 133 adult patients with MDS or CMML, including all French-American-British (FAB) classification criteria and IPSS Intermediate-1, Intermediate-2, or high-risk prognostic scores. Patients were randomized 1:1 to receive INQOVI (35 mg decitabine and 100 mg cedazuridine) orally in Cycle 1 and decitabine 20 mg/m² intravenously in Cycle 2 or the reverse sequence. Both INQOVI and intravenous decitabine were administered once daily on Days 1 through 5 of the 28-day cycle. Starting with Cycle 3, all patients received INQOVI orally once daily on Days 1 through 5 of each 28-day cycle until disease progression or unacceptable toxicity. No stratification was performed. Twenty-seven (20%) of the 133 patients went on to stem cell transplantation following INQOVI treatment.

The baseline demographic and disease characteristics are shown in [Table 6](#).

Table 6: Demographics and Baseline Disease Characteristics for Study ASTX727-02

Characteristic	N = 133
Age (years)	
Median (min, max)	71 (44, 88)
Sex (%)	
Male	65
Female	35

Characteristic	N = 133
Race (%)	
White	91
Black or African American	3
Asian	2
Other or Not Reported	4
ECOG Performance Score (%)	
0	41
1	59
Disease Category / IPSS (%)	
MDS INT-1	44
MDS INT-2	20
MDS High Risk	16
MDS Low Risk	8
CMML	12
Prior HMA Therapy* (%)	
Prior Azacitidine	5
Prior Decitabine	3
Transfusion Dependence[†] (%)	
RBC Transfusion Dependence	39
Platelet Transfusion Dependence	8

* One cycle only, per the Exclusion Criteria.

[†] Defined as documentation of ≥ 2 units of transfusion within 56 days prior to the first day of study treatment.

The primary outcome measure was comparison of the 5-day cumulative decitabine AUC between INQOVI and intravenous decitabine [see [Clinical Pharmacology \(11.3 Pharmacokinetics\)](#)]. Efficacy was established on the basis of complete response (CR) and the rate of conversion from transfusion dependence to transfusion independence. Efficacy results are shown in [Table 7](#). The median follow-up

time was 12.6 months (range: 9.3 to 20.5) and median treatment duration was 8.2 months (range 0.2 to 19.7).

Table7: Efficacy Results in Patients with MDS or CMML from Study ASTX727-02

Efficacy Endpoints	INQOVI (N = 133)
Complete Response (%) (95% CI)	21 (15, 29)
Median Duration of CR - months (range)*	7.5 (1.6, 17.5)
Median Time to CR - months (range)	4.3 (2.1, 15.2)

* From start of CR until relapse or death.

Among the 57 patients who were dependent on RBC and/or platelet transfusions at baseline, 30 (53%) became independent of RBC and platelet transfusions during any 56-day post-baseline period. Of the 76 patients who were independent of both RBC and platelet transfusions at baseline, 48 (63%) remained transfusion-independent during any 56-day post-baseline period.

Acute Myeloid Leukemia

INQOVI was evaluated in a Phase 3 (ASTX727-02 EU, NCT03306264) open-label, randomized, 2-cycle, 2-sequence crossover study that included 87 adult patients with de novo or secondary acute myeloid leukemia (AML) as defined by WHO criteria, who were not candidates for standard induction chemotherapy. Patients were randomized 1:1 to receive INQOVI (35 mg decitabine and 100 mg cedazuridine) orally in Cycle 1 and decitabine (20 mg/m²) intravenously in Cycle 2 or the reverse sequence. Both INQOVI and intravenous decitabine were administered once daily on Days 1 through 5 of the 28-day cycle. Starting with Cycle 3, all patients received INQOVI orally once daily on Days 1 through 5 of each 28-day cycle until disease progression, death, or unacceptable toxicity.

Demographic and baseline disease characteristics are shown in [Table 8](#).

Table 8: Demographic and Disease Baseline Characteristics (Phase 3)

Characteristic	Phase 3 [TRADEMARK] (N = 87)
Age (years)	
Median (min, max)	78 (61, 92)
Gender (%)	
Male	53 (60.9)
Female	34 (39.1)
ECOG Performance Score (%)	

Characteristic	Phase 3 [TRADEMARK] (N = 87)
0	36 (41.4)
1	51 (58.6)
Disease Category (%)	
de Novo AML	55 (63.2)
Secondary AML	32 (36.8)
MDS	18 (20.7)
Other antecedent hematological disorder	7 (8.0)
Therapy-related AML	7 (8.0)
Prior HMA Therapy (%)	
Prior Azacitidine	2 (2.3)
Prior Decitabine	0
Transfusion Dependence^a (%)	
RBC Transfusion Dependence	37 (42.5)
Platelet Transfusion Dependence	14 (16.1)

^a Defined as documentation of ≥ 2 units of transfusion within 56 days of the first day of study treatment.

Efficacy was established based upon complete response (CR) (see [Table 9](#)) and the rate of conversion from transfusion dependence to transfusion independence.

The primary outcome measure of the Phase 3 study was 5-day cumulative decitabine AUC between INQOVI and IV decitabine. INQOVI achieved AUC_{0-24hr} exposures equivalent to IV infusion of decitabine at 20 mg/ m² [see [Clinical Pharmacology \(11.3 Pharmacokinetics\)](#)].

The median treatment duration was 5 months (range 0 to 18 months).

Table 9: Efficacy Results in Patients with AML Study ASTX727-02-EU AML (Phase 3)

Efficacy Endpoints	INQOVI (N=87)
Complete Response (%) [95% CI]	22 [13.7, 32.0]
Median Duration of CR* - months [95% CI]	5.8 [3.3, NE]
Median Time to CR - months [range]	3.0 [1.8, 7.4]
Overall Response [†] (%) [95% CI]	32 [23, 43]

* From start of CR until relapse or death

[†] OR included patients with a best response of CR, CRi, and PR

A patient was considered transfusion independent if the patient was both red blood cell (RBC) and platelet transfusion free post-treatment for ≥ 56 consecutive days. Among the 41 patients who were dependent on either RBC and/or platelet transfusions at baseline, 14 (34%) became independent of RBC or platelet transfusions during any 56-day post-baseline period. Of the 46 patients who were independent

of both RBC and platelet transfusions at baseline, 12 (26%) remained transfusion-independent during any 56-day post-baseline period.

14 HOW SUPPLIED/STORAGE AND HANDLING

How Supplied

INQOVI tablets are biconvex, oval-shaped, film-coated, red, and debossed with “H35” on one side.

The tablets are packaged in Alu/Alu foil blisters and supplied as follows:

- 5 tablets in one blister card in a child-resistant carton

Storage and Handling

Store at or below 30°C. Dispense medication in the original packaging.

INQOVI is a cytotoxic agent. Follow applicable special handling and disposal procedures.

15 PATIENT COUNSELING INFORMATION

Myelosuppression

Advise patients of the risk of myelosuppression and to report any symptoms of fever, infection, anemia, or bleeding to their healthcare provider as soon as possible. Advise patients for the need for laboratory monitoring [see [Warnings and Precautions \(5.1 Myelosuppression\)](#)].

Embryo-Fetal Toxicity

Advise pregnant women of the potential risk to a fetus. Advise females of reproductive potential to inform their healthcare provider of a known or suspected pregnancy [see [Warnings and Precautions \(5.2 Embryo-Fetal Toxicity\)](#), [Use in Specific Populations \(8.1 Pregnancy\)](#)].

Advise females of reproductive potential to use effective contraception during treatment with INQOVI and for 6 months after the last dose [see [Use in Specific Populations \(8.3 Females and Males of Reproductive Potential\)](#)].

Advise males with female partners of reproductive potential to use effective contraception during treatment with INQOVI and for 3 months after the last dose [see [Use in Specific Populations \(8.3 Females and Males of Reproductive Potential\)](#), [Nonclinical Toxicology \(12.1 Carcinogenesis, Mutagenesis, Impairment of Fertility\)](#)].

Lactation

Advise women not to breastfeed during treatment with INQOVI and for 2 weeks after the last dose [*see [Use in Specific Populations \(8.2 Lactation\)](#)*].

Administration

Advise patients to take INQOVI at approximately the same time each day on an empty stomach. Instruct patients to avoid eating for at least 2 hours before and 2 hours after taking INQOVI. Advise patients on what to do when a dose is missed or vomited [*see [Dosage and Administration \(2.2 Recommended Dosage\)](#)*].

PRODUCT UNDER LICENCE FROM:

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